

1. Administrative & Study Identification

Full Study Title: A Randomized, Open-Label, Multicenter Phase III Study Evaluating Efficacy and Safety of Mosunetuzumab in Combination With Polatuzumab Vedotin in Comparison With Rituximab in Combination With Gemcitabine Plus Oxaliplatin in Participants With Relapsed or Refractory Aggressive B-Cell Non-Hodgkin's Lymphoma. **Short Title/Acronym:** SUNMO. **Protocol Number:** G043643. **NCT Number:** NCT05171647. **Posting ID:** 382738273488343476. **Sponsor Name:** Hoffmann-La Roche / Roche. **Investigational Product:** Mosunetuzumab (subcutaneous) + Polatuzumab vedotin (intravenous) [Mosun-Pola]. **Phase of Development:** Phase III. **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of mosunetuzumab in combination with polatuzumab vedotin (M+P) versus the standard of care regimen rituximab, gemcitabine, and oxaliplatin (R-GemOx) by assessing primary endpoints of Progression-Free Survival (PFS) and Objective Response Rate (ORR). **Secondary Objectives:** To evaluate investigator-assessed Objective Response Rate (ORR), Duration of Response (DOR), and Overall Survival (OS).

2.2 Background & Rationale To address the unmet medical need in patients with relapsed or refractory (R/R) aggressive non-Hodgkin lymphomas (aNHL), predominantly diffuse large B-cell lymphoma (DLBCL), high-grade B-cell lymphoma, transformed follicular lymphoma (trFL), and FL Grade 3B, who are ineligible for autologous stem cell transplant (ASCT) or who have relapsed after ASCT/CAR-T therapy. The combination of mosunetuzumab (an off-the-shelf CD20xCD3 T-cell engaging bispecific antibody) and polatuzumab vedotin (a CD79b-targeted antibody-drug conjugate) represents a novel fixed-duration, chemotherapy-free alternative aimed at providing superior efficacy and a manageable safety profile.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional. **Control Type:** Active Comparator (R-GemOx). **Blinding:** Open-label. **Randomization:** 2:1 ratio (Mosun-Pola vs. R-GemOx). **Trial Status:** Active, Not Recruiting.

3.2 Planned Enrollment & Duration Target **\$N\$:** 242 evaluated patients. **Number of Sites:** Estimated 75 sites globally. **Estimated Study Start:** [DD-MMM-YYYY] **Estimated Primary Completion:** [DD-MMM-YYYY]

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2. 2. CD20+ aggressive lymphoma determined by local hemopathology (e.g., DLBCL NOS, high-grade B-cell lymphoma, transformed follicular lymphoma, or Grade 3b FL). 3. Relapsed or refractory to at least one prior systemic therapy for aNHL. Participants with only one prior line of therapy must be ineligible for ASCT. 4. Measurable disease and adequate hepatic, hematologic, and renal function (CrCl ≥ 30 mL/min by Cockcroft-Gault or institutional standard). 5. Negative HIV test at screening. (Participants with a positive HIV test are eligible if stable on anti-retroviral therapy for at least 4 weeks, CD4 count ≥ 200 microliters, undetectable viral load, and no history of opportunistic infections within 12 months).

4.2 Exclusion Criteria 1. Pregnant or breastfeeding, or intending to become pregnant during the study or within specified months post-treatment. 2. Prior treatment with mosunetuzumab, other CD20-directed bispecific antibodies, R-GemOx, Gem-Ox, or prior treatment with polatuzumab vedotin (except those with documented response and no PD within 12 months, or ≤ 2 doses as bridging to CAR-T). 3. ASCT within 100 days, prior treatment with CAR-T cell therapy within 30 days, prior allogeneic stem cell transplant (SCT), or history of solid organ transplantation. 4. Significant cardiovascular disease (NYHA Class III/IV, MI within 6 months, unstable arrhythmias/angina) or significant active pulmonary disease/interstitial lung disease (or history of ILD/pneumonitis within 6 months). 5. Grade > 1 peripheral neuropathy or Grade > 1 persistent toxicity related to prior anti-lymphoma treatment. 6. Received anti-lymphoma treatments with monoclonal antibodies, radio-immunoconjugates, ADCs, or chemotherapeutic agents within 4 weeks (or 5 half-lives) prior to the first dose. Radiotherapy within 2 weeks or investigational therapy within 7 days prior to treatment. 7. Known or suspected history of hemophagocytic lymphohistiocytosis (HLH), progressive multifocal leukoencephalopathy (PML), severe allergic reactions to monoclonal antibody therapy, or history of other malignancy. 8. Current or past history of central nervous system (CNS) involvement of lymphoma, or CNS disease (e.g., stroke, epilepsy, CNS vasculitis, or neurodegenerative disease, with protocol-allowed exceptions). 9. Known active bacterial, viral, fungal, mycobacterial, parasitic, or other infection requiring IV antibiotics/hospitalization within 2 weeks. Known/suspected chronic active EBV, positive chronic Hepatitis B, acute/chronic Hepatitis C, or positive SARS-CoV-2 test within 7 days prior to enrollment. 10. History of autoimmune disease, or clinically

significant liver disease (e.g., viral hepatitis, cirrhosis). 11. Recent major surgery within 4 weeks, or administration of a live, attenuated vaccine within 4 weeks before the first dose. 12. Inability to comply with protocol-mandated activity restrictions or contraindication to any component of the study treatment.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Mosunetuzumab administered subcutaneously in combination with Polatuzumab vedotin administered intravenously. Control arm consists of Rituximab, Gemcitabine (e.g., Gemzar, Infugem), and Oxaliplatin (R-GemOx). **Route of Administration:** Subcutaneous (SC) + Intravenous (IV) for the experimental arm. **Duration of Treatment:** Fixed-duration outpatient therapy.

5.2 Concomitant & Prohibited Therapy Permitted: Intravenous tocilizumab as needed to manage cytokine release syndrome (CRS) events. **Prohibited:** Treatment with any other anti-lymphoma agent (investigational or otherwise) within 4 weeks (or 5 half-lives), or radiotherapy within 2 weeks prior to the first dose of study treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Lymphoma Biopsy/Scans, ECOG assessment, Labs, HIV/Viral Testing
Baseline	Day 0	Randomization, First Dose Administration
Interim	Treatment Cycles	Safety Labs, Efficacy Assessment, CRS Monitoring, PRO Questionnaires
End of Study	Follow-up	Final Scans, Survival Follow-up, AE Resolution Tracking

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) and Objective Response Rate (ORR). **Measurement Method:** Centrally assessed by an Independent Review Facility (IRF).

7.2 Secondary Endpoints [Endpoint 1] Objective response rate (ORR) as determined by the investigator. [Endpoint 2] Duration of response (DOR). [Endpoint 3] Overall survival (OS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring during the study period; specifically structured to identify and manage

Cytokine Release Syndrome (CRS) and Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS). **Serious Adverse Events (SAEs):** Standard reporting timeline, typically within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal and external committee details per Hoffmann-La Roche guidelines to review global Phase III safety data.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy endpoints (PFS, ORR). Safety Set for patients receiving at least one dose of study treatment. **Sample Size Justification:** ~242 patients, randomized 2:1, powered to detect statistically significant improvements in PFS and ORR. **Handling of Missing Data:** Standard censoring applied for time-to-event outcomes (PFS, OS, DOR); non-evaluable responses counted as non-responders for ORR.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote monitoring visits to ensure protocol adherence and safety tracking. **Audit Trail:** Centralized eCRF systems with audit logging for data clarification and independent radiological review queries.

11. Study Identifiers & Governance

Primary ID: G043643 **Registry Number:** NCT05171647 **Posting ID:** 382738273488343476 **Sponsor/Lead Organization:** Hoffmann-La Roche / Roche **Study Title:** SUNMO **Official Regulatory Title:** A Randomized, Open-Label, Multicenter Phase III Study Evaluating Efficacy and Safety of Mosunetuzumab in Combination With Polatuzumab Vedotin in Comparison With Rituximab in Combination With Gemcitabine Plus Oxaliplatin in Participants With Relapsed or Refractory Aggressive B-Cell Non-Hodgkin's Lymphoma **Trial Status:** Active, Not Recruiting

12. Clinical Framework (Lymphoma Specific)

Primary Condition: Relapsed or Refractory Aggressive B-Cell Non-Hodgkin's Lymphoma (R/R aNHL) **Diagnosis Threshold:** Local hemopathology confirmation of CD20+ disease **Medical Methodology:** Fixed-duration, chemotherapy-free targeted bispecific antibody + antibody-drug conjugate therapy **Optimization Target:** Improvement of PFS, ORR, and tolerability over standard R-GemOx

13. Study Procedures & Methodology

Management Pathway: Targeted therapeutic pathway for R/R aNHL.
Education Protocol (HCP): Site training focused on the identification and mitigation of Cytokine Release Syndrome (CRS), including proper administration of IV tocilizumab. **Education Protocol (Patient):** Informed consent education on potential side effects, including signs of CRS. **Quality Audit Mechanism:** Independent Review Facility (IRF) for centralized, unbiased assessment of radiological responses and PFS.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up continues until disease progression or end of study timeline (maturity of OS data). **Onsite Visit Cadence:** Concurrently with treatment cycles and defined tumor assessment intervals. **Remote Monitoring Frequency:** Periodic safety monitoring per site and protocol guidelines. **Checklist Review Interval:** Regular imaging intervals for PFS tracking as defined in the protocol.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				